



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متكرر

إسم المريض ربيعاً : كريمة ابراهيم ابراهيم حسن

المحافظة : القاهرة الحي / المدينة : حلوان

العنوان : 6 ش الشيخ حلوان البلد
حلوان

تاريخ تسجيل الإستمارة : 24-09-2026

رقم البطاقة : 27506250103363 رقم التليفون : 01129503090

العمر (تاريخ الميلاد) : 25-06-1975 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: كريمة ابراهيم ابراهيم حسن National ID: 27506250103363	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الى Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Handwritten Arabic text, possibly a signature or name, consisting of several loops.

Handwritten Arabic text, possibly a signature or name, consisting of several loops.

Handwritten Arabic text, possibly a signature or name, with a large '18' written below it.





02/22/2026

MDT FORM

Name:	Karima Ibrahim Ibrahim Hsna	MRN:	5390
Age:	51 Y	ID:	
Menopausal Sta	pre	Phone:	01129503090
NRS SCORE:		Marital Status:	
BMI:		Family History:	-ve
CO Morbidity:	free	Reason Of MDT:	

Clinical Data:

BX: IDC GII
ER: 8/8 PR: 8/8 HER2: +1 KI67:

R

L

Sonomammography:

Mets Workup:

History:

CT4b N2 Mo

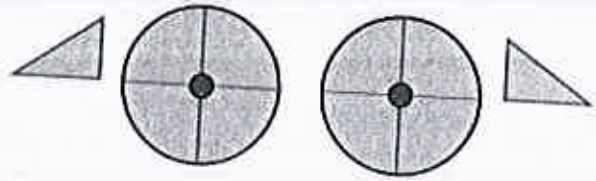
pt received NACT AC*4 TXT*4 >>> It MRM >>> PCR >> PORT

pt on Tam , zoladex since 5-2025 >> to be shifted to AI + zoladex 10.8 mg

Recommendations:

MDT Outcome & Plan

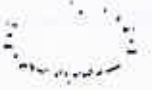
shift to AI + zoladex 10.8 mg



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان عزت		د. تومين رمضان			



Patient Name : الأستاذ / كريمه ابراهيم ابراهيم حسن
Patient No. : 27506250103363
Path No. : 1882/24
Sex : Female
Age : 48 Y
Referring Doc. :

Request No. : 13003357
Request Date : 03/06/2024
Verification Date : 01/07/2024
Financial Category : اورام دار السلام - مبادئ
Department : Out Patient
IHC No. :
Registered by : Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 3/6/2024.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 70% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+).
- Ki67: 45% nuclear reaction of tumor cells.

Conclusion:

Left breast swelling, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: (score 1+).
- Ki67: Positive in 45% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :

Printing Date : 20/07/2024 10:22:25

Page 1 of 2

Printed By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

Patient Name الأستاذة / كريمة إبراهيم إبراهيم حسن
Patient No. 27506250103363
Path No. 1882/24
Sex Female
Age 48 Y
Referring Doc.

Request No. 13003357
Request Date 03/06/2024
Verification Date 01/07/2024
Financial Category اورام دار السلام - مبادره
Department Out Patient

Registered by Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Comment: Although Her2 is Negative, Such cases (Score 1+) are predictive of response to Her2-Ab drug conjugates (ADCs) according to latest 2023 ASCO-CAP guidelines.

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :

Printing Date : 20/07/2024 10:22:25

Page 2 of 2

Printed By : Mrs. Mayada Hegazy

CAP
ACCREDITED

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name : الأستاذ / كريمه ابراهيم ابراهيم حسن
Patient No. : 27506250103363
Path No. : 1882/24
Sex : Female
Age : 48 Y
Referring Doc :

Request No. : 13003357
Request Date : 03/06/2024
Verification Date : 12/06/2024
Financial Category : اورام دار السلام - مبانره
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Left breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 3/06/2024.

Gross Examination:

- Three grayish white tissue cores up to 3.5 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angioinvasion: No.

Diagnosis:

Left breast swelling, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :

Printing Date : 20/07/2024 10:22:29

Page 1 of 1

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذ / كريمه ابراهيم ابراهيم حسن
Patient No. : 27506250103363
Path No. : 1118/25
Sex : Female
Age : 49 Y
Referring Doc :

Request No. : 13001724
Request Date : 22/03/2025
Verification Date : 13/04/2025
Financial Category : اورام دار السلام - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer post NACT .
Nature of the specimen: Left breast & Left axilla (MRM)

Gross Examination:

- Procedure: Left MRM measured 28x20x8 cm with covering skin ellipse measured 22x10 cm .
- Localization: UOQ
- Tumor extent : A greyish white tumour bed site
- Size: 3x3 x1 cm .
- Skin involvement: Nipple & areola were unremarkable grossly .
- Distance from margins: 3 cm away from deep margin
- Microscopic examination:
- Histological type: Invasive duct carcinoma (Cellularity 65%)
- Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1)
- Ductal carcinoma in situ: Focal (5%), cribriform
- Skin involvement: Free nipple and areola
- Lymph node involvement: Free L.Ns (0/10)
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic partial response

Diagnosis:

- Left MRM (post NACT) :
Invasive duct carcinoma grade II
- Free margins
 - Free nipple and areola
 - Free L.Ns (0/10)
 - Pathologic partial response
- Stage : ypT2, ypN0, yp Mx

Verified by :

Dr.Heba Hasan

Printing Date : 16/04/2025 12:34:16

Page 1 of 2

Printing By : Mrs.Mayada Hegazy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt





Patient Name : الأستاذة / كريمة ابراهيم ابراهيم حسن	Request No. : 13001724
Patient No. : 27506250103363	Request Date : 22/03/2025
File No. : 1118/25	Verification Date : 13/04/2025
Sex : Female	Financial Category : اورام دار السلام - مبلره
Age : 49 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

• RCB : 1.555 , RCB class II

Signature
Prof. Dr.Lubna Omar
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

Printing Date : 16/04/2025 12:34:16

Page 2 of 2

Printing By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

PATIENT NAME: KARIMA IBRAHIM IBRAHIM HASSAN**GENDER:** Female**DATE :** 25-Jul-2024 01:08 PM**PATIENT ID:** 10124246029**AGE:** 49 y**ORDER:** 2959308,2959309**CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY****INDICATION:**

- Left IDC

TECHNIQUE:

- Contrast Enhanced Spectral Mammography CC and MLO views after IV non-ionic contrast medium infusion (100 ml)
- Complementary US

REPORT:

- Contrast Enhanced Spectral Mammography showed minimal symmetrical back ground parenchymal enhancement.
- Left 11 O'clock heterogeneously enhancing mass measuring about 36x33x31 mm associated with 11 and 12 O'clock adjacent similar but smaller masses measuring about 9 and 12 mm respectively. The farthest one is seen about 30 mm from the index mass, 10 mm from the overlying skin and about 57 mm from the nipple. Associated diffuse left skin thickening reaching about 5.5 mm.
- No right breast mass or non mass enhancement.
- Normal right skin thickness and contour.
- Normal nipple areola complexes.
- Two pathologically enlarged left axillary (level I) lymph nodes with effaced hilum; the larger measuring about 21x16 mm.
- Right non-specific axillary lymph nodes.
- No pathologically enlarged left supraclavicular lymph nodes

OPINION:

- Locally advanced left breast multifocal malignant masses with pathological ipsilateral axillary lymph nodes (BIRADS 6).
- No right breast pathological mass or non mass enhancement (BIRADS 1)

Reported By,**Dr. Sally Fouad , MD****25-Jul-2024 01:32 PM**



Patient Name: Kafima Ibrahim Ibrahim Hassan	Date : 31/07/2024
Exam : C.T CHEST WITH CONTRAST	Age : 49 Year(s)
Medical ID: 2292/2024	Pat. ID: 76937

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS:

I Chest:

- Left breast UIQ parenchymal soft tissue lesion measures about 2.3 x 2.8 cm along its axial dimensions. Enlarged ipsilateral axillary (level I) lymph nodes, the largest one seen at level I measures about 1.5 x 2 cm in axial dimensions.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & Pelvis:

- The liver is enlarged with homogenous CT texture. No sizable focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Under filling of the urinary bladder showing minimal circumferential mural thickening.
- Bulky uterus for age, for US correlation.



Patient Name: Karima Ibrahim Ibrahim Hassan	Date : 31/07/2024
Exam : C.T CHEST WITH CONTRAST	Age : 49 Year(s)
Medical ID: 2292/2024	Pat. ID: 76937

- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- D12 and smaller L2 vertebral bodies faint sclerotic lesions are seen., the former measuring about 15x 14 mm.

Opinion:

- Left breast neoplastic parenchymal soft tissue lesion with metastatic ipsilateral axillary lymph nodes.
- • D12 and smaller L2 vertebral bodies faint sclerotic lesions for bone scan correlation.
- No sizable pulmonary or, hepatic metastatic deposits.

Dr. Reem ELwaly

Dr. Ali Elsisy, MSc.

Signature Prof. Menna Tallah Hanafy,
12/08/2024 07:45 م

PATIENT NAME: KARIMA IBRAHIM IBRAHIM HASSAN

GENDER: Female

DATE : 02-Sep-2024 01:45 PM

PATIENT ID: 10124246029

AGE: 49 y

ORDER: 3016934

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.

Degenerative changes are seen over dorso-lumbar vertebrae.

Arthritic changes are seen over both knees.

Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

02-Sep-2024 02:16 PM

PATIENT: KARIMA IBRAHIM IBRAHIM HASSAN, 10124246029, Female, 49 y

Report version 1, 02-Sep-2024 02:16 PM

Page 1 of 1

www.Baheya.org

16602

مركز بهية الهرم
4 شارع طلحة شارع الهرم بجوار دوي. الحيزة
مستشفى بهية الشيخ زايد

Bone Scan
[[DOCTYPE:BONE_SCAN]]



pp

وزارة الصحة والمكان
امقة المراكز الطبية المتخصصة
مستشفى اورام دار السلام

Echo report

Name: كريمة ابراهيم
Date: ٢٠/٧ 2025

Age:

٢٢٩٢
٢٤

EF	<u>65%</u>	54-74%	PWT	<u>0.8</u>	0.6-1.1cm
LVEDD	<u>4.9</u>	3.5-5.7cm	AO	<u>2.3</u>	2.0-3.7cm
LVESD		2.2-4.0cm	LA	<u>3.5</u>	1.9-4.0cm
FS		25-45%	RV		0.7-2.3cm (supine)
SWT	<u>0.9</u>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions: cmLV wall thickness: cmLV diastolic function: DD grade ISWMA. cmLA size cmAortic root dimension cmMitral Valve cmAortic valve cmTricuspid valve cmPulmonary valve cmRight side size and systolic function cmIntracardiac masses and thrombi cmPericardium cmIVC diameter and collapsibility. cm

Conclusion:

D/